

CLINICAL MULTIPLE CHOICE EVALUATION

LECTURE 6 OF 20: WOUND CARE PATHOPHYSIOLOGY AND ASSESSMENT



Candidate Instructions

- This textbook-grounded paper contains **25 strict Multiple Choice Questions** matching your course curriculum.
- All content is explicitly derived from the provided material, covering skin anatomy, wound etiologies, TIMERS, and the TILI framework.
- Select the single best choice (**A, B, C, or D**) for each question item.
- The full verification key matrix is cleanly isolated at the absolute end of this document.

QUESTION 1

According to Slide 5, which description perfectly matches the structural traits and role of the Dermis?

- A) The outermost layer that provides a barrier and creates skin tone, containing Merkel cells
- B) The middle layer that supports and nourishes the epidermis, containing collagen/elastin fibres, blood vessels, nerves, hair follicles, and sweat glands
- C) The deepest layer composed of pure fat cells designed to insulate the body and absorb severe shock
- D) The sterile layer that regulates body flora pH levels via immediate commensal colonization

QUESTION 2

Why are the skin structures of infants and young children physically more prone to blistering and swelling?

- A) Because their skin is entirely sterile and lacks commensal flora protection
- B) Because they possess a weaker attachment between the epidermis and dermis due to flatter rete pegs
- C) Because their collagen helix structure undergoes a sudden collapse at birth
- D) Because their hypodermis lacks connective tissue fibers required to absorb friction shock

QUESTION 3

What specific structural degeneration characterizes the skin during the standard biological aging process?

- A) The rete pegs become significantly more elongated, creating an inflexible skin boundary
- B) The skin thins and lines as the structural collagen helix collapses
- C) The commensal flora drops the surface pH level down to an acidic 2.5 benchmark
- D) The epidermal cells lose their keratinocytes, preventing any vitamin D synthesis

QUESTION 4

Which of the following contains the correct sequence of events that characterize the immediate 'Haemostasis' phase of injury response?

- A) Blood vessels dilate, phagocytes migrate, and chemical mediators clear the surface debris
- B) Fibroblasts produce thick type I collagen strands while new capillaries form via angiogenesis
- C) Blood vessels constrict, platelet plug formation occurs, and the clotting cascade is activated
- D) Epidermal cells flatten, lose their cytoplasm, and form an unalterable corneal scar tissue barrier

QUESTION 5

The 'Inflammation' phase of injury response begins within minutes to hours. What specific white blood cells migrate to the site to clear debris?

- A) Keratinocytes and Melanocytes
- B) Neutrophils and Macrophages
- C) Langerhans cells and Merkel cells
- D) Fibroblasts and Endothelial tracking lines

QUESTION 6

During the 'Proliferation' phase of healing, what is the explicit role of Angiogenesis?

- A) The constriction of dermal capillaries to maximize central core body homeostasis
- B) The formation of new blood vessels to deliver oxygen and nutrients to injured areas and promote repair
- C) The systematic breakdown of nonviable cells via localized phagocytic enzymatic action
- D) The reorganization of fibrous tissue arrays from type III collagen into firm type I units

QUESTION 7

What physiological events take place during the Maturation (Remodelling) phase of a wound?

- A) Damaged cells release chemical mediators within minutes to activate a sterile environment
- B) Fibroblasts initiate immediate platelet plug formation over the open epidermis borders
- C) Collagen fibres reorganise, tensile strength increases, and scar tissue forms over weeks to months
- D) Flatter rete pegs develop underneath the tissue bed to re-establish normal skin tone settings

QUESTION 8

Match the intentions of wound healing with their correct definitions. Which statement is perfectly accurate according to Slide 11?

- A) Primary intention describes leaving a wound completely open to fill in and close naturally without stitches
- B) Secondary intention refers to a wound where the edges are closely re-approximated via immediate primary closure
- C) Tertiary intention (delayed primary closure) occurs when a wound is initially left open after debridement of all nonviable tissue
- D) Primary intention refers to wounds packed daily with gauze to encourage slough formation over 3 months

QUESTION 9

What is the explicit duration threshold used in your lecture slides to define a "Chronic Wound"?

- A) A wound that fails to proceed through an orderly and timely reparative process over a period of 3 weeks
- B) A wound that fails to proceed through an orderly and timely reparative process over a period of 6 weeks
- C) A wound that fails to proceed through an orderly and timely reparative process over a period of 3 months
- D) A wound that fails to proceed through an orderly and timely reparative process over a period of 12 months

QUESTION 10

How does Slide 18 define an 'Abrasion' wound type, and what is its clinical characteristic?

- A) A clean, straight cut typically made by a surgical scalpel or a sharp piece of paper
- B) A superficial wound caused by friction or scraping against a rough surface; quite painful and common in young children
- C) A deep cut or tear caused by sharp glass that involves pre-tibial laceration tracking
- D) A small, deep penetration caused by a pointed animal nail, highly prevalent in the hands

QUESTION 11

Skin tears fall under the category of 'Lacerations'. What alternate title is given to skin tears across your curriculum slides?

- A) Superficial grazes
- B) Pre-tibial lacerations
- C) Degloving avulsions
- D) Contused hematomas

QUESTION 12

Where are puncture wounds caused by pointed objects piercing the skin described as being 'very common' in practice?

- A) The hands and forearms
- B) The feet
- C) The pre-tibial shin zone
- D) The sacral and bony prominences

QUESTION 13

What is the exact definition and alternate medical title given to an 'Avulsion' wound?

- A) A closed injury resulting from blunt force trauma, also known as a haematoma
- B) A wound where a portion of skin and sometimes underlying tissue is torn away; also known as a degloving injury
- C) A superficial scrape caused by intense cold, also known as frostbite
- D) An intentionally self-inflicted cut used to manage severe emotional distress

QUESTION 14

Under which clinical wound category do 'Haematomas' explicitly belong within your lecture materials?

- A) Puncture Wounds
- B) Avulsions
- C) Contusions (Bruises)
- D) Incisions

QUESTION 15

According to Slide 24, phone chargers and household appliances are cited as common examples behind skin damage caused by which source?

- A) Dry Heat burns
- B) Chemical burns
- C) Electricity burns
- D) Radiation burns

QUESTION 16

What core etiology defines a 'Crush Injury' as stated on Slide 27?

- A) Superficial scraping of the epidermal tissue against a rough industrial surface
- B) A wound caused by a heavy object compressing the body, leading to damage to skin, muscles, and bones
- C) Prolonged unrelieved vertical pressure over a single localized bony prominence
- D) The sudden exposure of un-colonized sterile skin to caustic cement or bitumen

QUESTION 17

Why does a nurse need to complete a comprehensive wound assessment? Which reason is explicitly stated to justify referrals and prevent resource waste?

- A) To document the absolute contraction rate of the hypodermis matrix every 24 hours
- B) To address the fact reported by Guest et al. (2020) that many wounds lack a differential diagnosis
- C) To re-establish a flat rete peg configuration across adult-like dermal fields
- D) To eliminate the presence of normal commensal flora around surgical lacerations

QUESTION 18

What does the complete acronym T.I.M.E.R.S stand for within the wound assessment framework?

- A) Timeline, Incision, Medication, Exudate, Recovery, Support
- B) Tissues, Infection, Moisture, Edges, Regeneration / repair, Social and patient related factors
- C) Tone, Inflammation, Macule, Epithelium, Rete pegs, Severity score
- D) Trauma, Ischaemia, Moisture, Eschar, Remodelling, Surgical plan

QUESTION 19

How is 'Granulation Tissue' explicitly described under the 'T is for Tissue' section?

- A) Yellow/white fibrous tissue composed of dead cells that accumulate inside dry eschar
- B) New connective tissue and microscopic blood vessels that form on the surfaces of a wound during the healing process
- C) A light pink layer with a shiny pearl appearance that travels from the outward wound edges
- D) Devitalized nonviable tissue that cannot be salvaged and requires immediate debridement

QUESTION 20

What specific characteristics and cellular makeup define 'Slough' in a wound bed?

- A) Light pink tissue with a shiny pearl appearance composed of crawling germinal cells
- B) Yellow/white material with a soft texture, consisting of dead cells that accumulate in the wound exudate
- C) Tightly packed keratinized epidermal cells growing directly from the base matrix
- D) Fibrous collagen configurations that represent an advanced regeneration pathway

QUESTION 21

The presence of Necrosis or Eschar indicates the absolute clinical need for which intervention?

- A) Immediate surgical closure by primary intention sutures
- B) Debridement (the removal of dead or devitalized tissue)
- C) Lactation induction using bioengineered technologies
- D) Application of oil-based sebum to moisten the corneal layers

QUESTION 22

How does 'Epithelial Tissue' explicitly manifest visually in a healing wound bed?

- A) Thick, rough, dark brown material tightly adhered to the base tendon structure
- B) As light pink with a shiny pearl appearance, crawling from the outward wound edges across the bed
- C) As uneven, dark purple granular clusters that bleed heavily with mild movement
- D) A dry, yellow, fibrous coating that prevents bacterial migration loops

QUESTION 23

According to Slide 38, when does scarring occur, and what type of process does scar tissue represent?

- A) Scarring occurs when normal tissue undergoes an autograft, representing an adaptive protection process
- B) Scarring occurs when 'regeneration' fails due to the amount of tissue destruction; scar tissue represents a 'repair' process
- C) Scarring occurs when platelets fail to plug the capillary beds, representing an inflammatory process
- D) Scarring occurs when commensal normal flora colonizes the dermis, representing a homeostatic process

QUESTION 24

The TILI (2021) framework serves as an evidence-based tool for localized infections. What core strategy does it prioritize?

- A) The immediate application of systemic high-dose heavy metal or arsenic therapies
- B) Early intervention as part of an antimicrobial stewardship (AMS)-focused approach to wound care
- C) Delaying documentation until after the full closure of secondary intention granulation tracks
- D) Bypassing Tissue Viability Specialist consults during high-occupancy ward events

QUESTION 25

The Nursing and Midwifery Council (NMC) emphasizes principles of good record keeping. What explicit directive is given regarding "Language" on Slide 46?

- A) Records should use local slang words to describe the patient's emotional bay status
- B) Records should be written in a language that's free of unnecessary abbreviations or jargon
- C) Records should prioritize pharmaceutical trade labels over generic entries across all lines
- D) Records must be written in Latin medical code strings to preserve information security lines

OFFICIAL EXAM ANSWER KEY MATRIX

Verify candidate choice selections strictly using this validation matrix block.

Q1	Q2	Q3	Q4	Q5	Q6	Q7	Q8	Q9	Q10
B	B	B	C	B	B	C	C	C	B

Q11	Q12	Q13	Q14	Q15	Q16	Q17	Q18	Q19	Q20
B	B	B	C	C	B	B	B	B	B

Q21	Q22	Q23	Q24	Q25
B	B	B	B	B